

NWPharmaTech CHR-P initiative

Public programme brief | Version 2026-09-20

Informational overview for visitors, collaborators and press.

Not an offering document. Not medical advice.

1. Mission

Intervene earlier in the psychosis pathway - supporting rigorous research aimed at people at clinical high risk while keeping pharmaceutical-sponsor accountability and clear evidence standards.

2. The need

CHR-P identifies elevated transition risk; it is not a schizophrenia diagnosis and does not mean transition is inevitable. There is still no broadly approved pharmacotherapy labelled to prevent psychosis in CHR-P across major markets.

3. Investigational programme

NWPT-SM32300 is NWPharmaTech's investigational 300 mg CBD micellar-emulsion softgel. The proposed CHR-P programme would evaluate its effects on symptoms and functioning.

Effectiveness in this population has not been established. Prevention or delay of transition to psychosis is a longer-term question that must be separately powered if claimed as primary. Endpoints and design are being locked in a version-controlled synopsis.

4. Evidence status (summary)

- Field CHR-P literature: elevated, heterogeneous risk - see evidence library.
- Other CBD products: hypothesis-generating only - not NWPT-SM32300 efficacy.
- Programme-specific efficacy / prevention: not established.
- Early BA work: numerical claims withheld until a signed clinical study report is available.

5. Research roadmap

Qualitative horizons only (no invented progress percentages):

- Now: public informational science pages with clear field vs programme labelling.
- Near: version-controlled clinical synopsis; signed early-study reports before any numerical PK/safety claims.
- Near: prepare a rigorous evaluation of symptom and functioning outcomes in defined CHR-P populations.
- Later: larger transition-to-psychosis objectives only if separately powered and financed.

6. Governance

NWPharmaTech remains pharmaceutical sponsor. Investigators, ethics committees and regulators retain independent roles. Community or financing participants do not vote on protocol, dose, endpoints, safety or regulatory submissions.

7. Financing note

Approximately \$10 million is a working fundraising target for planning - not committed capital, a validated trial budget, or a priced offer. This site does not accept investments.

8. Contact

Press and general enquiries: team@nwpharmatech.com.

Do not send clinical or personal health information by email.

Selected field references

1. Fusar-Poli P, Bonoldi I, Yung AR, et al. Predicting psychosis: meta-analysis of transition outcomes in individuals at high clinical risk. Arch Gen Psychiatry. 2012;69(3):220-229. doi:10.1001/archgenpsychiatry.2011.1472
2. Yung AR, Yuen HP, McGorry PD, et al. Mapping the onset of psychosis: the Comprehensive Assessment of At-Risk Mental States. Aust N Z J Psychiatry. 2005;39(11-12):964-971. doi:10.1080/j.1440-1614.2005.01714.x
3. Cannon TD, Yu C, Addington J, et al. An individualized risk calculator for research in prodromal psychosis. Am J Psychiatry. 2016;173(10):980-988. doi:10.1176/appi.ajp.2016.15070890
4. Salazar de Pablo G, et al. Probability of Transition to Psychosis in Individuals at Clinical High Risk: An Updated Meta-analysis. JAMA Psychiatry. 2021;78(9):970-978. doi:10.1001/jamapsychiatry.2021.0830
5. McGuire P, Robson P, Cubala WJ, et al. Cannabidiol (CBD) as an Adjunctive Therapy in Schizophrenia: A Multicenter Randomized Controlled Trial. Am J Psychiatry. 2018;175(3):225-231. doi:10.1176/appi.ajp.2017.17030325 (other product / schizophrenia adjunct - NOT NWPT-SM32300, NOT CHR-P prevention).

Full citations and links: <https://www.nwpharmatech.org/evidence.html>

Confidential investor diligence materials are not published in this brief.

Corporate identity: <https://nwpharmatech.com> | Programme site: <https://www.nwpharmatech.org>